

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	Tisagenlecleucel Versus Standard of Care in Adult Patients With Relapsed or Refractory Aggressive B-cell Non-Hodgkin Lymphoma: A Randomized, Open Label, Phase III Trial (BELINDA)
Short Title:	Tisagenlecleucel in Adult Patients With Aggressive B-cell Non-Hodgkin Lymphoma
Protocol Number:	301400121054714299
NCT Number:	NCT03570892
Sponsor Name:	Novartis
Investigational Product:	tisagenlecleucel
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Event-free Survival (EFS) Per Blinded Independent Review Committee (BIRC) Assessment: Event-free survival (EFS) is defined as the time from the date of randomization to the date of the first documented disease progression or stable disease at or after the week 12 (+/- 1 week) assessment, as assessed by Blinded Independent Review Committee (BIRC) per Lugano criteria, or death due to any cause, at any time.
Secondary Obj.:	1. Event Free Survival (EFS) as Assessed by Local Investigator 2. Overall Survival (OS) 3. Overall Response Rate (ORR)

2.2 Background & Rationale

This is a randomized, open label, multicenter phase III trial comparing the efficacy, safety, and tolerability of tisagenlecleucel to Standard Of Care in adult patients with aggressive B-cell Non-Hodgkin Lymphoma after failure of rituximab and anthracycline containing frontline immunochemotherapy.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE3, Status: ACTIVE_NOT_RECRUITING
Target N:	331

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

* Histologically confirmed, aggressive B-cell NHL at relapse/progression or PR after front line therapy. Aggressive B-cell NHL is heretofore defined by the following list of subtypes (Swerdlow et al 2016):

* DLBCL, NOS,

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- * FL grade 3B,
- * Primary mediastinal large B cell lymphoma (PMBCL),
- * T cell rich/histiocyte rich large B cell lymphoma (T/HRBCL),
- * DLBCL associated with chronic inflammation,
- * Intravascular large B-cell lymphoma,
- * ALK+ large B-cell lymphoma,
- * B-cell lymphoma, unclassifiable, (with features intermediate between DLBCL and classical Hodgkin's Lymphoma (HL)),
- * High grade B-cell lymphoma with MYC and BCL2 and/or BCL6 rearrangements,
- * High-grade B-cell lymphoma, NOS
- * HHV8+ DLBCL, NOS
- * DLBCL transforming from follicular lymphoma
- * DLBCL transforming from marginal zone lymphoma
- * DLBCL, leg type
- * Relapse or progression within 365 days from last dose of anti CD20 antibody and anthracycline containing first line immunochemotherapy or refractory (have not achieved a CR).
- * Patient is considered eligible for autologous HSCT as per local investigator assessment. Note: Intention to transplant and type of high dose chemotherapy (HDCT) regimen will be documented at the time of study entry
- * Disease that is both active on PET scan (defined as 5-Deauville scorepoint-scale of 4 or 5) and measurable on CT scan, defined as::
- * Nodal lesions ≥ 15 mm in the long axis, regardless of the length of the short axis, and/or
- * Extranodal lesions (outside lymph node or nodal mass, but including liver and spleen) ≥ 10 mm in long AND short axis
- * Eastern Cooperative Oncology Group (ECOG) performance status 0 or 1
- * Adequate organ function:

Renal function defined as:

- * Serum creatinine of $\leq 1.5 \times$ upper limit of normal (ULN), OR estimated glomerular filtration rate (eGFR) ≥ 60 mL/min/1.73 m²

Hepatic function defined as:

- * Alanine Transaminase (ALT) and Aspartate Transaminase (AST) $\leq 5 \times$ ULN
- * Total bilirubin $\leq 1.5 \times$ ULN with the exception of patients with Gilbert syndrome who may be included if their total bilirubin is $\leq 3.0 \times$ ULN and direct bilirubin $\leq 1.5 \times$ ULN

Hematologic Function (regardless of transfusions) defined as:

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- * Absolute neutrophil count (ANC) $\geq 1000/\text{mm}^3$
- * Absolute lymphocyte count (ALC) $\geq 300/\text{mm}^3$ OR Absolute number of CD3+ T cells $\geq 150/\text{mm}^3$ (only for patients with non-historical apheresis)
- * Platelets $\geq 50000/\text{mm}^3$
- * Hemoglobin $\geq 8.0 \text{ g/dl}$

Adequate pulmonary function defined as:

- * No or mild dyspnea (? Grade 1)
- * Oxygen saturation measured by pulse oximetry $\geq 90\%$ on room air
- * Forced expiratory volume in 1 s (FEV1) $\geq 50\%$ and/or carbon monoxide diffusion test (DLCO) $\geq 50\%$ of predicted level - Must have a leukapheresis material of non-mobilized cells available for manufacturing.

4.2 Exclusion Criteria

- * Prior treatment with anti-CD19 therapy, T cell therapy, or any prior gene therapy product
- * Treatment with any systemic lymphoma-directed second line anticancer therapy prior to randomization. Only steroids and local irradiation are permitted for disease control
- * Patients with active central nervous system (CNS) involvement by disease under study are excluded, except if the CNS involvement has been effectively treated and local treatment was ≥ 4 weeks before randomization
- * Prior allogeneic HSCT
- * Clinically significant active infection
- * Any of the following cardiovascular conditions:
 - * Unstable angina, myocardial infarction, coronary artery bypass graft (CABG), or stroke within 6 months prior to screening,
 - * Left ventricle ejection fraction (LVEF) $< 45\%$ as determined by echocardiogram (ECHO) or magnetic resonance angiography (MRA) or multigated acquisition (MUGA) at the screening assessment.
 - * New York Heart Association (NYHA) functional class III or IV (Chavey et al 2001), within the past 12 months.
 - * Clinically significant cardiac arrhythmias (e.g., ventricular tachycardia), complete left bundle branch block, high-grade atrioventricular (AV) block (e.g., bifascicular block, Mobitz type II) and third degree AV block unless adequately controlled by pacemaker implantation.
 - * Resting QTcF $\geq 450 \text{ msec}$ (male) or $\geq 460 \text{ msec}$ (female) at screening or inability to determine the QTcF interval
 - * Risk factors for Torsades de Pointes (TdP), including uncorrected hypokalemia or hypomagnesemia, history of cardiac failure, or history of clinically significant/ symptomatic bradycardia, or any of the following:
 - * Long QT syndrome, family history of idiopathic sudden death or congenital long QT syndrome
 - * Concomitant medication(s) with a "Known Risk of Torsades de Pointes" per crediblemeds.org that cannot be discontinued or replaced by safe alternative medication.
 - * Patients with active neurological autoimmune or inflammatory disorders (e.g., Guillain-Barré Syndrome (GBS), Amyotrophic Lateral Sclerosis (ALS)) and clinically significant active cerebrovascular disorders (e.g. cerebral edema, posterior reversible encephalopathy syndrome (PRES))

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11. Study Identifiers & Governance

Primary ID: 301400121054714299
Registry Number: NCT03570892
Sponsor: Novartis
Study Title: Tisagenlecleucel in Adult Patients With Aggressive B-cell Non-Hodgkin Lymphoma

15. Sign-off & Approval

Principal Investigator: _____ Date: _____
Clinical Operations Lead: _____ Date: _____
Lead Statistician: _____ Date: _____